FLOX: fluorouracil, folinic acid and oxaliplatin

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 1, 15 and 29: oxaliplatin + fluorouracil/folinic acid

Active medicines: Oxaliplatin + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally once	Before the oxaliplatin-containing dose Review QT risk, interactions, constipation and hepatic function.
Dexamethasone	8 mg orally once	Before the oxaliplatin-containing dose Reconcile all steroid prescriptions; do not duplicate.
Dexamethasone	8 mg orally once daily	Days 2 and 3 after the oxaliplatin-containing dose Confirm the prescribed delayed prophylaxis and re-anchor it if treatment is delayed.
Metoclopramide	10 mg orally up to three times daily as required	Rescue only; duration to be completed by prescriber Review renal function, neurological contraindications and interactions.

Additional phase-specific support

- Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.
- Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- Avoid cold exposure as advised after oxaliplatin. Report persistent numbness or difficulty with buttons, walking or everyday tasks before the next dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____

Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00486 v8 · NCCP NCIS antiemetics V8, Table 1; apply oral regimen guidance separately

Sources checked 2026-11-04 · SACTCheck v0.84.32 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Days 8, 22 and 36: fluorouracil/folinic acid only

Active medicines: Folinic acid + Fluorouracil

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before the fluorouracil-only treatment visit National Cancer Control Programme (NCCP) NCIS V8 low-risk default. Review QT risk, interactions, constipation and hepatic function; clinician may select an alternative.
Metoclopramide	10 mg orally up to three times daily as required	Rescue; prescriber confirms duration Check renal function, neurological contraindications and interactions.

Additional phase-specific support

- Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.
- Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.
- Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.
- Avoid cold exposure as advised after oxaliplatin. Do not suck ice chips during this regimen.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____

Prescriber / date: _____ Pharmacy review: _____

NCCP 00486 v8 · NCCP NCIS antiemetics V8

Sources checked 2026-11-04 · SACTCheck v0.84.32 · Regimen-level drafts; prescriber and oncology-pharmacy verification required